Patient Record — ID 1693

Age	33	**Sex**	F
Race/Ethnicity	Non-Hispanic White	**Insurance**	Commercial PPO
Total Encounters	8	**Generation Seed**	3381

Primary Diagnoses

• Conservative management of placenta previa with bed rest
• Preterm labor
• Placenta percreta
• Placenta previa
• Placenta accreta
• Stress urinary incontinence
• Placenta previa (complete)

Comorbidities

• History of previous cesarean delivery
• Multiparity

Patient Profile

Sex	F
Age at First Encounter	33
Race/Ethnicity	Non-Hispanic White
Insurance	Commercial PPO
Occupation	Elementary school teacher
Smoking Status	Never
Alcohol Use	Occasional (1-2 drinks per month)

Allergies:

• No known drug allergies

Chronic Conditions:

• History of gestational diabetes mellitus
• Stress urinary incontinence

Home Medications:

• Metformin - 500 mg oral twice daily
• Prenatal vitamin - 1 tablet daily

Surgical History:

• Low transverse cesarean section x2
• Dilation and curettage

Family History:

• Type 2 diabetes mellitus (mother)
• Preeclampsia (sister)

Encounters (8)

Encounter 1 — 2020-01-15 — Obstetrics & Gynecology

Encounter ID	6915	**Type**	outpatient
Attending	Dr. Sarah Chen	**Order**	0
Department	Obstetrics & Gynecology	**Gen. Method**	template

Chief Complaint:

Six-month history of intermittent vaginal wetness during exercise or sneezing.

Full Note:

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ENCOUNTER NOTE
Date: 2020-01-15
Provider: Dr. Sarah Chen, MD
Department: Obstetrics & Gynecology
Visit Type: outpatient

CHIEF COMPLAINT:
Six-month history of intermittent vaginal wetness during exercise or sneezing.

HISTORY OF PRESENT ILLNESS:
She presents to the clinic with a 6-month history of intermittent vaginal wetness, particularly
during exercise or when she sneezes. She denies any recent sexual activity and has no history of
urinary tract infections.

PAST MEDICAL HISTORY:
Active Problem List:
- History of gestational diabetes mellitus
- Stress urinary incontinence

Her obstetric history is significant for two vaginal deliveries, the most recent being 18 months
ago.

PAST SURGICAL HISTORY:
- Low transverse cesarean section x2
- Dilation and curettage

MEDICATIONS:
Home Medications:
- Metformin 500 mg oral twice daily
- Prenatal vitamin 1 tablet daily

ALLERGIES:
No known drug allergies

FAMILY HISTORY:
- Type 2 diabetes mellitus (mother)
- Preeclampsia (sister)

SOCIAL HISTORY:
Smoking: Never. Alcohol: Occasional (1-2 drinks per month). Occupation: Elementary school teacher.

PHYSICAL EXAMINATION:
On physical examination, no abnormalities are noted at rest. However, upon performing the Valsalva
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maneuver, no fluid is observed emerging from the cervix. A speculum examination is performed, and the nitrazine test is negative. Ferning is not observed on microscopic examination of the vaginal fluid.

Encounter 2 — 2021-01-15 — Obstetrics & Gynecology

Encounter ID	6914	**Type**	inpatient
Attending	Dr. Michael Torres	**Order**	1
Department	Obstetrics & Gynecology	**Gen. Method**	hybrid

Chief Complaint:

Presented in active labor with contractions every 3 minutes; underwent vaginal delivery complicated by retained placenta and hemorrhage.

Full Note:

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ENCOUNTER NOTE
Date: 2021-01-15
Provider: Dr. Michael Torres, MD
Department: Obstetrics & Gynecology
Visit Type: inpatient

CHIEF COMPLAINT:
Presented in active labor with contractions every 3 minutes; underwent vaginal delivery
complicated by retained placenta and hemorrhage.

HISTORY OF PRESENT ILLNESS:
The patient is a 34-year-old female with gestational diabetes mellitus managed on metformin and a
history of stress urinary incontinence who presents in active labor. She was evaluated one year
prior for intermittent vaginal wetness with exertion consistent with this history. Her pregnancy
has been uncomplicated, and she has received routine prenatal care. After several hours of labor,
she delivers a healthy male infant vaginally. However, the placenta does not deliver spontaneously
after 30 minutes, and manual attempts at removal are unsuccessful. The patient begins to
experience significant vaginal bleeding. The bleeding continues despite conservative management,
including uterine massage and oxytocin administration.

PAST MEDICAL HISTORY:
Active Problem List:
- Stress urinary incontinence (diagnosed 2020-01-15)
- History of gestational diabetes mellitus

PAST SURGICAL HISTORY:
She has a history of two prior cesarean sections.

MEDICATIONS:
Home Medications:
- Metformin 500 mg oral twice daily
- Prenatal vitamin 1 tablet daily

ALLERGIES:
No known drug allergies

FAMILY HISTORY:
- Type 2 diabetes mellitus (mother)
- Preeclampsia (sister)

SOCIAL HISTORY:
Smoking: Never. Alcohol: Occasional (1-2 drinks per month). Occupation: Elementary school teacher.

VITAL SIGNS:
On examination, her vital signs are stable. Her blood pressure is 100/60 mmHg, and her heart rate
is 110 bpm.

PHYSICAL EXAMINATION:
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On examination, the fetal heart tracing is reassuring. On examination, the uterus is firm, and the cervix is closed.

LABORATORY DATA:
The patient's hemoglobin drops from 12.5 g/dL to 9.8 g/dL over the next hour.

IMAGING:
An urgent ultrasound shows an abnormally adherent placenta with no clear plane between the placenta and the myometrium.

Encounter 3 — 2023-01-15 — Obstetrics & Gynecology

Encounter ID	6916	**Type**	outpatient
Attending	Dr. Jennifer Walsh	**Order**	2
Department	Obstetrics & Gynecology	**Gen. Method**	hybrid

Chief Complaint:

Routine prenatal visit at 28 weeks gestation; asymptomatic.

Full Note:

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ENCOUNTER NOTE
Date: 2023-01-15
Provider: Dr. Jennifer Walsh, MD
Department: Obstetrics & Gynecology
Visit Type: outpatient

CHIEF COMPLAINT:
Routine prenatal visit at 28 weeks gestation; asymptomatic.

HISTORY OF PRESENT ILLNESS:
She returns for prenatal care at 28 weeks with history of gestational diabetes, retained placenta,
and stress incontinence. Her pregnancy remains uncomplicated; she is compliant with prenatal
vitamins and routine care and denies vaginal bleeding, contractions, or fluid leakage.

PAST MEDICAL HISTORY:
Active Problem List:
- Stress urinary incontinence (diagnosed 2020-01-15)
- Placenta accreta (diagnosed 2021-01-15)
- History of gestational diabetes mellitus

Her past medical history is significant for gestational diabetes during her first pregnancy, which
was diet-controlled.

PAST SURGICAL HISTORY:
- Low transverse cesarean section x2
- Dilation and curettage

MEDICATIONS:
Home Medications:
- Metformin 500 mg oral twice daily
- Prenatal vitamin 1 tablet daily

ALLERGIES:
No known drug allergies

FAMILY HISTORY:
- Type 2 diabetes mellitus (mother)
- Preeclampsia (sister)

SOCIAL HISTORY:
Smoking: Never. Alcohol: Occasional (1-2 drinks per month). Occupation: Elementary school teacher.

VITAL SIGNS:
Her blood pressure is 120/78 mmHg

PHYSICAL EXAMINATION:
Her physical examination is unremarkable

IMAGING:
A routine prenatal ultrasound is performed, which reveals that the placenta is covering the
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internal cervical os.

Encounter 4 — 2023-02-15 — Obstetrics & Gynecology

Encounter ID	6909	**Type**	outpatient
Attending	Dr. Jennifer Walsh	**Order**	3
Department	Obstetrics & Gynecology	**Gen. Method**	template

Chief Complaint:

Follow-up for complete placenta previa diagnosed at 28 weeks; instructed on pelvic rest and activity restriction.

Full Note:

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ENCOUNTER NOTE
Date: 2023-02-15
Provider: Dr. Jennifer Walsh, MD
Department: Obstetrics & Gynecology
Visit Type: outpatient

CHIEF COMPLAINT:
Follow-up for complete placenta previa diagnosed at 28 weeks; instructed on pelvic rest and
activity restriction.

PAST MEDICAL HISTORY:
Active Problem List:
- Stress urinary incontinence (diagnosed 2020-01-15)
- Placenta accreta (diagnosed 2021-01-15)
- Placenta previa (complete) (diagnosed 2023-01-15)
- History of gestational diabetes mellitus

PAST SURGICAL HISTORY:
- Low transverse cesarean section x2
- Dilation and curettage

MEDICATIONS:
Home Medications:
- Metformin 500 mg oral twice daily
- Prenatal vitamin 1 tablet daily

ALLERGIES:
No known drug allergies

FAMILY HISTORY:
- Type 2 diabetes mellitus (mother)
- Preeclampsia (sister)

SOCIAL HISTORY:
Smoking: Never. Alcohol: Occasional (1-2 drinks per month). Occupation: Elementary school teacher.
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Encounter 5 — 2023-03-15 — Emergency Medicine

Encounter ID	6912	**Type**	ed
Attending	Dr. Priya Patel	**Order**	4
Department	Emergency Medicine	**Gen. Method**	hybrid

Chief Complaint:

Sudden-onset, painless vaginal bleeding while at home.

Full Note:

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ENCOUNTER NOTE
Date: 2023-03-15
Provider: Dr. Priya Patel, MD
Department: Emergency Medicine
Visit Type: ed

CHIEF COMPLAINT:
Sudden-onset, painless vaginal bleeding while at home.

HISTORY OF PRESENT ILLNESS:
The patient presents to the emergency department with sudden-onset, painless vaginal bleeding in
the setting of known complete placenta previa managed with pelvic rest and activity restriction.
She denies abdominal pain, contractions, or recent trauma.

PAST MEDICAL HISTORY:
Active Problem List:
- Stress urinary incontinence (diagnosed 2020-01-15)
- Placenta accreta (diagnosed 2021-01-15)
- Placenta previa (complete) (diagnosed 2023-01-15)
- Conservative management of placenta previa with bed rest (diagnosed 2023-02-15)
- History of gestational diabetes mellitus

Her previous deliveries were all by cesarean section.

PAST SURGICAL HISTORY:
- Low transverse cesarean section x2
- Dilation and curettage

MEDICATIONS:
Home Medications:
- Metformin 500 mg oral twice daily
- Prenatal vitamin 1 tablet daily

ALLERGIES:
No known drug allergies

FAMILY HISTORY:
- Type 2 diabetes mellitus (mother)
- Preeclampsia (sister)

SOCIAL HISTORY:
Smoking: Never. Alcohol: Occasional (1-2 drinks per month). Occupation: Elementary school teacher.

PHYSICAL EXAMINATION:
On examination, she is hemodynamically stable, fetal heart tones are normal, and the uterus is
soft and non-tender. Speculum exam reveals active bleeding from the cervical os, but digital
cervical exam is deferred.

IMAGING:
Transabdominal ultrasound shows a placenta covering the internal cervical os.
```

PLAN:
She is admitted for monitoring and stabilization.

Encounter 6 — 2023-04-15 — Emergency Medicine

Encounter ID	6913	**Type**	ed
Attending	Dr. David Kim	**Order**	5
Department	Emergency Medicine	**Gen. Method**	hybrid

Chief Complaint:

Recurrent painless vaginal bleeding described as sudden gush while standing; denies contractions or trauma.

Full Note:

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ENCOUNTER NOTE
Date: 2023-04-15
Provider: Dr. David Kim, MD
Department: Emergency Medicine
Visit Type: ed

CHIEF COMPLAINT:
Recurrent painless vaginal bleeding described as sudden gush while standing; denies contractions
or trauma.

HISTORY OF PRESENT ILLNESS:
The patient returns with recurrent painless vaginal bleeding in the setting of known complete
placenta previa, following a similar presentation approximately one month prior, describing
bleeding that began suddenly this morning. She says she was standing in her kitchen when she felt
a gush of blood, but has had no contractions, abdominal pain, or trauma. The bleeding has since
slowed but has not stopped completely. She denies recent intercourse and has had no vaginal exams
recently. Fetal movements are normal.

PAST MEDICAL HISTORY:
Active Problem List:
- Stress urinary incontinence (diagnosed 2020-01-15)
- Placenta accreta (diagnosed 2021-01-15)
- Placenta previa (complete) (diagnosed 2023-01-15)
- Conservative management of placenta previa with bed rest (diagnosed 2023-02-15)
- Placenta previa (diagnosed 2023-03-15)
- History of gestational diabetes mellitus

Her two prior deliveries were by cesarean section.

PAST SURGICAL HISTORY:
- Low transverse cesarean section x2
- Dilation and curettage

MEDICATIONS:
Home Medications:
- Metformin 500 mg oral twice daily
- Prenatal vitamin 1 tablet daily

ALLERGIES:
No known drug allergies

FAMILY HISTORY:
- Type 2 diabetes mellitus (mother)
- Preeclampsia (sister)

SOCIAL HISTORY:
Smoking: Never. Alcohol: Occasional (1-2 drinks per month). Occupation: Elementary school teacher.

VITAL SIGNS:
Vital signs are stable and fetal heart rate tracing is reactive.
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PHYSICAL EXAMINATION:
Physical exam shows a soft, non-tender uterus without contractions. There is visible blood in the vaginal vault, but no active bleeding. A sterile speculum exam is performed, but digital cervical examination is deferred.

PLAN:
Transabdominal ultrasound is ordered.

Encounter 7 — 2023-05-15 — Obstetrics & Gynecology

Encounter ID	6910	**Type**	inpatient
Attending	Dr. Lisa Thompson	**Order**	6
Department	Obstetrics & Gynecology	**Gen. Method**	hybrid

Chief Complaint:

Regular painful contractions every 5 minutes for 2 hours at 35 weeks gestation.

Full Note:

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ENCOUNTER NOTE
Date: 2023-05-15
Provider: Dr. Lisa Thompson, MD
Department: Obstetrics & Gynecology
Visit Type: inpatient

CHIEF COMPLAINT:
Regular painful contractions every 5 minutes for 2 hours at 35 weeks gestation.

HISTORY OF PRESENT ILLNESS:
The patient with gestational diabetes, stress incontinence, placental accreta, and placenta previa
presents. She is currently at 35 weeks gestation with an uncomplicated pregnancy and appropriate
prenatal care.

PAST MEDICAL HISTORY:
Active Problem List:
- Stress urinary incontinence (diagnosed 2020-01-15)
- Placenta accreta (diagnosed 2021-01-15)
- Placenta previa (complete) (diagnosed 2023-01-15)
- Conservative management of placenta previa with bed rest (diagnosed 2023-02-15)
- Placenta previa (diagnosed 2023-03-15)
- History of gestational diabetes mellitus

She reports that her previous delivery was via cesarean section due to fetal distress.

PAST SURGICAL HISTORY:
- Low transverse cesarean section x2
- Dilation and curettage

MEDICATIONS:
Home Medications:
- Metformin 500 mg oral twice daily
- Prenatal vitamin 1 tablet daily

ALLERGIES:
No known drug allergies

FAMILY HISTORY:
- Type 2 diabetes mellitus (mother)
- Preeclampsia (sister)

SOCIAL HISTORY:
Smoking: Never. Alcohol: Occasional (1-2 drinks per month). Occupation: Elementary school teacher.

VITAL SIGNS:
On examination, her vital signs are stable,

PHYSICAL EXAMINATION:
Fetal heart tones are normal. The cervix is dilated to 4 cm, and the fetus is in a breech
presentation. There is no evidence of placental abruption or fetal distress.
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Encounter 8 — 2023-08-15 — Obstetrics & Gynecology

Encounter ID	6911	**Type**	inpatient
Attending	Dr. Sarah Chen	**Order**	7
Department	Obstetrics & Gynecology	**Gen. Method**	hybrid

Chief Complaint:

Admitted for elective repeat cesarean delivery at 38 weeks gestation.

Full Note:

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ENCOUNTER NOTE
Date: 2023-08-15
Provider: Dr. Sarah Chen, MD
Department: Obstetrics & Gynecology
Visit Type: inpatient

CHIEF COMPLAINT:
Admitted for elective repeat cesarean delivery at 38 weeks gestation.

HISTORY OF PRESENT ILLNESS:
Patient with prior placenta accreta and recent bleeding presents for elective cesarean delivery at
38 weeks. During delivery, the infant is delivered without difficulty; however, the placenta
cannot be removed manually. The umbilical cord stretches but does not detach, and brisk vaginal
bleeding follows. The uterus remains soft despite uterotonic administration.

PAST MEDICAL HISTORY:
Active Problem List:
- Stress urinary incontinence (diagnosed 2020-01-15)
- Placenta accreta (diagnosed 2021-01-15)
- Placenta previa (complete) (diagnosed 2023-01-15)
- Conservative management of placenta previa with bed rest (diagnosed 2023-02-15)
- Placenta previa (diagnosed 2023-03-15)
- Preterm labor (diagnosed 2023-05-15)
- History of gestational diabetes mellitus

Her obstetric history includes two prior cesarean sections and one dilation and curettage
procedure after a spontaneous abortion.

PAST SURGICAL HISTORY:
- Low transverse cesarean section x2
- Dilation and curettage

MEDICATIONS:
Home Medications:
- Metformin 500 mg oral twice daily
- Prenatal vitamin 1 tablet daily

ALLERGIES:
No known drug allergies

FAMILY HISTORY:
- Type 2 diabetes mellitus (mother)
- Preeclampsia (sister)

SOCIAL HISTORY:
Smoking: Never. Alcohol: Occasional (1-2 drinks per month). Occupation: Elementary school teacher.
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